

Orthopaedic Recovery Protocol

Post-Surgical Recovery for Joint Replacement and Fracture Repair | City General Hospital

1. Overview

This protocol covers post-discharge recovery for patients who have undergone orthopaedic surgery including total hip/knee replacement, fracture fixation (ORIF), and related bone or joint procedures.

The goals in the immediate post-discharge period are to prevent wound infection and DVT, restore safe mobility through physiotherapy, and manage pain effectively. Your physiotherapy programme is as important as your medications.

2. Post-Surgery Mobility Guidelines

- Follow your physiotherapist's weight-bearing instructions precisely; do not deviate.
- Use mobility aids (crutches, frame) as instructed; do not discard them prematurely.
- Hip replacement: do not bend the hip > 90° or cross legs for 6 weeks.
- Knee replacement: full extension exercises are the highest priority in weeks 1–2.
- Walk short distances every 1–2 hours rather than one long walk; promotes circulation.

3. Weight-Bearing Status by Procedure

Procedure	Initial Status	Full Weight-Bearing
Total Hip Replacement	Weight-bearing as tolerated	Day 1–3 with frame
Total Knee Replacement	Weight-bearing as tolerated	Day 1–3 with frame
Femur Fracture — IM Nail	Non-weight-bearing	6–12 weeks post-op
Hip Fracture — ORIF	Partial weight-bearing	Per surgeon instruction
Distal Radius Fracture (cast)	Normal arm use with cast	After cast removal
Arthroscopy (diagnostic)	Full weight-bearing day 0	Immediate

Always follow your surgeon's specific written instructions over these general guidelines.

4. Wound Care

Orthopaedic wounds carry a higher infection risk due to the presence of metalwork. Strict wound hygiene and early detection of infection signs are essential.

- Keep wound dry for the first 7 days; use a waterproof cover when showering.
- Do not apply creams or lotions to the wound unless directed by your surgical team.
- Staples/sutures are removed at your day-7 wound check; do not remove them yourself.
- Purulent (cloudy/yellow/green) discharge, increasing redness, or fever > 38 °C: report same day.

5. Pain Management

Adequate pain control is essential for physiotherapy participation. Under-treated pain causes reduced mobility and worse outcomes.

- Take analgesics at regular intervals as prescribed; do not wait for severe pain.
- Paracetamol 1 g four times daily is the safest base analgesic — take with or without food.
- NSAIDs (ibuprofen): take with food; avoid if you have kidney disease or heart failure.
- Opioids (tramadol): do not drive; taper off as pain improves within 7–10 days.
- Apply ice (15–20 min, 3×/day over a cloth) for joint swelling in the first 2 weeks.

6. Deep Vein Thrombosis Prevention

Major orthopaedic surgery carries a high DVT risk. Medication + physical measures together reduce this risk by over 60%.

- Continue anticoagulant (rivaroxaban/enoxaparin) for the full prescribed duration (14–35 days).
- Wear compression stockings continuously for the first 2 weeks; wash alternate pairs daily.
- Ankle pump exercises: flex/extend the foot 10 times every hour while awake.
- Stay well hydrated — at least 2 litres of water daily to reduce blood viscosity.

7. Physiotherapy Milestones

Time Point	Mobility Goal	Exercise Focus	Aid Required
Day 1–3	Walk to toilet + short corridor	Ankle pumps, static quads	Frame
Week 1	Walk 50–100 m independently	Seated knee bends, leg raises	Frame / crutches
Week 2	Manage stairs with rail	Hip abduction, knee extension	One crutch
Week 4	Walk 200–400 m; household tasks	Step-ups, balance exercises	Stick if needed
Week 6	Outpatient physio assessment	Pool therapy, light gym	Usually unaided
3 Months	Return to most daily activities	Return-to-sport assessment	Unaided

8. Warning Signs — Infection and Blood Clots

- Report the following immediately. Deep infections and pulmonary embolism are life-threatening if untreated.
 - Increasing wound redness, warmth, swelling, or pus beyond 48 hours post-discharge.
 - Fever > 38 °C persisting beyond day 2 post-operatively.
 - Unilateral calf pain, redness, or swelling — classical DVT signs.
 - Sudden breathlessness, chest pain, or coughing blood — pulmonary embolism; call 112.

9. Follow-Up Schedule

Appointment	Purpose	Action if Missed
Day 7	Wound check + suture removal	Contact orthopaedic outpatients immediately
Week 2	Physiotherapy review	Contact physiotherapy department
Week 6	Surgeon review + X-ray	Book earliest available slot
3 Months	Function + implant review	Contact orthopaedic secretary

For urgent out-of-hours concerns contact the on-call orthopaedic registrar via the hospital switchboard. Synthetic document — demo only.